SMART INDIA HACKATHON · 2026

Swasthya*Setu* AI

Rural Healthcare Accessibility & Quality — closing the gap between a symptom at home and the right care, at the right facility.

PROBLEM STATEMENT ID	THEME	PS CATEGORY
26133	MedTech / HealthTech	Software

TEAM
TECH MINDS
ID · 119557

A digital bridge connecting patients, ASHA workers & Primary Health Centres.

01 — THE PROBLEM, THE IDEA

A patient's journey shouldn't break at the clinic door.

What's broken today

01 Patients, ASHA workers and PHCs work in isolation — there is no shared digital thread linking them.

02 Deciding whether a symptom needs a PHC visit, a specialist, or can wait, is often guesswork.

03 Care stops at the consultation — follow-up and continuity are left to memory, not a system.

04 Referral notes still travel on paper, and get lost in the handoff between facilities.

What SwasthyaSetu AI does

Unified Healthcare Ecosystem

Connects patients, ASHA workers and Primary Health Centres into a single digital network.

AI-Guided Multilingual Triage

Runs a quick preliminary symptom assessment in the patient's own language to guide the care decision.

Smart Queue & Referral Tracking

Real-time queue updates and seamless referral handoffs, so no patient drops off between tiers of care.

02 — TECHNICAL APPROACH

Two flows, one shared spine.

Patient / Family flow

- **Understand the need**
  Patient or family identifies a health concern.
- **Contact ASHA**
  Shares symptoms, receives initial guidance.
- **Visit nearby PHC**
  Guided to the nearest facility if needed.
- **Referred onward**
  Escalated to a higher facility when required.
- **Follow-up & continuity**
  Reminders keep the patient in the loop.

Doctor flow

- **Secure login**
  Doctor signs in to SwasthyaSetu AI.
- **Today's patients**
  Scheduled and walk-in list for the day.
- **AI patient summary**
  History and prior visits at a glance.
- **Consultation**
  Examines and discusses with the patient.
- **Treatment & follow-up**
  Prescription given, next check-in scheduled.

Why a shared spine

Both flows write to the same patient record. When the doctor sets a follow-up, it becomes the ASHA worker's next reminder — nothing is re-typed, nothing is re-explained.

STACK

Frontend	Backend	Database	Security	Deployment
React + Tailwind CSS	Supabase (BaaS, auth, realtime)	PostgreSQL	Role-based access, row-level security	Vercel / Netlify

03 — FEASIBILITY & VIABILITY

Built to work with what already exists.

Modular & scalable

Patient, appointment, referral and follow-up sit as separate modules — each can ship, and grow, on its own timeline.

Low infrastructure dependency

Runs on the smartphones and computers already in use at PHCs and ASHA centres — no specialised hardware.

WHERE THE DESIGN EARNS ITS KEEP

Challenge — deciding the right level of care

AI-assisted triage

Points each patient toward PHC, district hospital, or specialist — instead of a guess.

Challenge — care ends at the consultation

Follow-up alerts

Automatic reminders to patient and ASHA worker keep continuity from slipping.

Challenge — paper records lost in transfer

Digital referral queue

A single digital record moves with the patient — nothing to misplace at the handoff.

04 — IMPACT & BENEFITS

What changes when the thread doesn't break.

NO PATIENT LOST BETWEEN FACILITIES

Connected care

Referral, treatment and follow-up stay linked across the public healthcare system — a patient handed off between facilities is never handed off blind.

RIGHT SUPPORT, RIGHT PATIENT

Targeted care

Prioritises ASHA and frontline support for the patients who need it most, instead of spreading it thin.

CARE CLOSER TO HOME

Stronger primary care

Raises visibility and use of the nearby PHC before a distant facility becomes the default answer.

Earlier access · Fewer unnecessary visits · Better continuity of care · Improved accessibility

05 — RESEARCH & REFERENCES

Grounded in what already runs at scale.

01 **Ayushman Arogya Mandir Portal**
Ministry of Health & Family Welfare

02 **Digitally Enabled Care**
Industry case study

03 **EHR Standards for India**
nrces.in — digital health & EHR interoperability

04 **eSanjeevani**
esanjeevani.mohfw.gov.in — existing national teleconsultation portal

05 **Healthcare Resource Guide, India**
Current analysis of rural healthcare access

06 **Global Innovation Benchmark**
MIT Solve — global health innovation case studies

Thank you.